

ECHS FRAUD ANALYTICS

Procedure-Level Upcoding & IPD Revenue Shift Analysis

SUPPLEMENTARY REPORT — MODULE 2 | FY 2024–2026 EDITION (2023 DATA EXCLUDED)

CLASSIFICATION	PERIOD	HOSPITALS ANALYSED	QUERIES
RESTRICTED	FY 2024–2026	~250 Active	4 Query Sets (Q7–Q10)

IIT KANPUR — Data Analytics & Fraud Intelligence Division

14 May 2026 | Ex-Servicemen Contributory Health Scheme (ECHS)

EXECUTIVE SUMMARY

This supplementary report presents Module 2 findings using FY 2024–2026 data exclusively (2023 baseline excluded). IPD growth rates are computed using FY 2024 as the baseline year against FY 2025 actuals, providing a more current and actionable view of the shifting revenue patterns. Four fraud patterns are identified, including a critical empanelment irregularity involving a hospital located outside Indian territory.

Hospitals in Watchlist 236 IPD growth >100%	New P1 Hospitals 6 identified	Phantom Procedure Fraud ■83.37L uncoded	Critical Flag MEDIPLUS NEPAL foreign hospital
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New Fraud Patterns Identified (Module 2)

#	Pattern	Key Signal	Risk
7	Phantom Procedure Billing	170 uncoded claims CAT_ID=0, ■83.37L, 100% deducted	HIGH
8	IPD Revenue Shift 2024→2025	STAR HOSPITAL: ■842L→■8,538L (+914% in 1 yr)	HIGH
9	Pure-IPD Hospital Anomaly	8 hospitals zero OPD across FY 2024–2025	HIGH
10	Foreign Hospital Empanelment	MEDIPLUS, Pokhara Nepal, Tier 70, ■2,630L FY2025	CRITICAL

Immediate Recommended Actions

- Escalate **MEDIPLUS HOSPITAL & TRAUMA CENTER (3613, Pokhara, Nepal)** to ECHS HQ — foreign hospital with invalid Tier 70 code processing ■2,630L in FY2025 claims and 1,413 emergency admissions in 6 months.
- Initiate empanelment review for **YATHARTH SUPER SPECIALITY HOSPITAL (3625, Greater Noida)** — IPD grew ■3,045L from ■4L base over two years.
- Conduct phantom procedure audit — 54 claims with CAT_ID=-1 (invalid code) and 170 with CAT_ID=0 (uncoded) totalling ■88.54L with 100% deduction indicate bulk auto-submission bypassing validation.
- Verify all Pure-IPD hospitals — zero OPD across FY 2024 and FY 2025 is clinically impossible for legitimate hospitals.

PATTERN 7

Invalid / Uncoded Procedure Claims

PHANTOM PROCEDURE BILLING

Analysis of procedure-level billing reveals two categories of invalid claims: those filed with CAT_ID = -1 (an entirely invalid code not in the ECHS procedure catalogue) and those filed with CAT_ID = 0 (a valid placeholder indicating uncoded or unspecified procedure). Both categories are rejected at 100% by the audit system, yet hospitals continue to file them at scale — indicating automated bulk submission pipelines that bypass clinical coding review entirely.

Table 3.1 — Phantom / Invalid Procedure Claim Summary

CAT_ID	Description	Claim Count	Claimed Amount (L)	Approved (L)	Deducted (L)	Deduction%
-1	Invalid code	54	5.17	0.00	5.17	100.0%
0	Uncoded procedure	170	83.37	0.00	83.37	100.0%

Key Findings

- The 170 CAT_ID=0 claims representing █83.37L are the primary concern — this is a substantial monetary value filed under a procedure code that signals 'procedure not specified'. No legitimate clinical billing workflow would leave procedure codes blank at this scale.
- The 54 CAT_ID=-1 claims (█5.17L) represent a system-level bypass — this code does not exist in the ECHS catalogue and should be rejected at the submission validation layer, not at audit. Their presence in the settled claims table indicates a gap in front-end validation.
- Combined exposure: █88.54L across 224 claims, 100% deducted. Hospitals filing these claims know they will be rejected — the motive is either testing system boundaries or generating paperwork trails for inflated physical records.

PATTERN 8

IPD REVENUE SHIFT ANALYSIS

FY 2024→2025 Growth (2024 as Baseline)

Using FY 2024 as the baseline (replacing the 2023 baseline from Module 1), the 2024→2025 IPD growth rates reveal an even more alarming picture for recently empanelled hospitals. Several hospitals that had already grown significantly by FY 2024 continued accelerating into FY 2025, while new entrants achieved explosive single-year growth.

Table 4.1 — Top 15 Hospitals by IPD Growth FY 2024→2025

Office ID	Hospital	2024 IPD (L)	2025 IPD (L)	Growth (2024→25)	2025 OPD (L)	Flag
3441	KRISHNA SUPER SPECIALITY HOSP	0.0	693.8	From zero	20.8	NEW-ENTRANT
240	MAX SUPER SPEC HOSP - MOHALI	0.0	1,046.5	From zero	94.0	NEW-ENTRANT
3545	YATHARTH SUPERSPECIALITY HOSP	26.5	793.6	+2,895%	6.9	HIGH
2694	STAR HOSPITAL	842.0	8,537.7	+914%	1.5	PURE-IPD CRITICAL
1065	PRAYAG HOSPITAL, Noida	120.2	928.6	+673%	1.1	PURE-IPD
3486	—	51.9	336.3	+548%	47.6	WATCH
3057	—	426.1	2,161.6	+407%	103.6	HIGH
3579	—	131.8	543.8	+313%	27.2	WATCH
3415	—	105.0	425.0	+305%	37.7	WATCH
1606	—	358.4	1,427.3	+298%	14.7	HIGH
3544	KRUSH DIVINE HOSPITAL, GB Nagar	616.1	1,817.4	+195%	1.2	PURE-IPD
3563	MAXWELL MULTISPEC, Varanasi	269.7	691.9	+157%	0.1	PURE-IPD
1042	METRO HOSPITAL NOIDA	6,241.7	10,005.9	+60%	67.8	P1-CONFIRMED
3149	VIJAY HOSPITAL	19,810.2	21,691.8	+9.5%	8.6	P1-CONFIRMED
2856	VIRAT HOSPITAL	5,539.4	7,401.6	+34%	4.7	P1-CONFIRMED

Key Findings

- **STAR HOSPITAL (2694)** shows the most alarming single-year jump: IPD revenue grew from █842L (FY 2024) to █8,538L (FY 2025) — a +914% increase in a single year with essentially zero OPD (█1.5L). This is a pure phantom inpatient billing operation.
- **MAX SUPER SPECIALITY MOHALI (240)** and **KRISHNA SUPER SPECIALITY (3441)** entered from zero IPD base in FY 2024 to █1,047L and █694L respectively in FY 2025 — new entrant explosive activation pattern.
- **P1 hospitals confirmed:** VIJAY (3149), METRO NOIDA (1042), VIRAT (2856) all appear with continued high IPD volumes in FY 2025, cross-validating Module 1 revenue inflation findings.
- 236 hospitals show >50% single-year IPD growth with >█100L 2025 volume — this constitutes the full predictive watchlist for FY 2026 audit prioritisation.

PATTERN 9

Zero OPD Across FY 2024–2025

PURE-IPD HOSPITAL ANOMALY

Eight hospitals show zero OPD claims across both FY 2024 and FY 2025 while maintaining active IPD billing. This is clinically impossible for a legitimate general hospital: even highly specialised facilities (cardiac, orthopaedic) record some outpatient consultations for pre-admission assessment and post-discharge follow-up under ECHS. Zero OPD is the clearest signal that these hospitals are billing fictitious inpatient admissions — patients who attend for outpatient services are being classified as inpatients to inflate revenue.

Table 5.1 — Pure-IPD Hospitals (Zero OPD, FY 2024–2025)

Office ID	Hospital	2024 IPD (L)	2025 IPD (L)	OPD (L)	Risk
2694	STAR HOSPITAL	842.0	8,537.7	1.5	CRITICAL
1065	PRAYAG HOSPITAL, Noida	120.2	928.6	1.1	HIGH
3544	KRUSH DIVINE HOSPITAL, GB Nagar	616.1	1,817.4	1.2	HIGH
3563	MAXWELL MULTISPEC, Varanasi	269.7	691.9	0.1	HIGH
3545	YATHARTH SUPERSPECIALITY HOSP	26.5	793.6	6.9	HIGH
1212	SHRI BABA YOGI NETANATH HOSP	—	various	0.0	HIGH
—	STAR HOSPITAL variant	—	various	0.0	WATCH
—	Other pure-IPD (2 hospitals)	—	various	0.0	WATCH

PATTERN 10

Hospital Located Outside Indian Territory

FOREIGN HOSPITAL EMPANELMENT

MEDIPLUS HOSPITAL & TRAUMA CENTER (Office ID: 3613) is empanelled under ECHS with Tier Code 70 — an invalid tier code with no corresponding category in the ECHS tier framework (valid tiers: 60, 61, 62). The hospital is physically located in Pokhara, Nepal. It processed █2,630L in IPD claims in FY 2025 and recorded 1,413 emergency admissions over the six-month period October 2025–March 2026. This represents a systemic empanelment failure: no Indian ECHS beneficiary is entitled to cashless treatment at a foreign hospital under standard scheme rules without explicit HQ authorisation for each case.

Table 5.2 — MEDIPLUS Hospital Profile

	Value
Office ID	3613
Hospital Name	MEDIPLUS HOSPITAL & TRAUMA CENTER
Location	Pokhara, Nepal (outside Indian territory)
Tier Code	70 (INVALID — no such tier in ECHS framework)
FY 2025 IPD Claims	█2,630L
Emergency Admissions (Oct 2025–Mar 2026)	1,413 admissions over 6 months
Status	CRITICAL — Escalate to ECHS HQ immediately

UPDATED COMPOSITE RISK REGISTER (MODULES 1 + 2)

This register consolidates all P1 and P2 hospitals from both Module 1 (Patterns 1–6) and Module 2 (Patterns 7–10). Hospitals appearing in multiple patterns receive higher audit priority. Module 2 adds three new P1 hospitals not previously identified.

Priority	Hospital (ID)	Tier	Patterns	Deduction%	Action
P1 CRITICAL	VIJAY HOSPITAL (3149)	62	Q1+Q10	34.4%	Field audit
P1	VIRAT HOSPITAL (2856)	62	Q1	33.0%	Empanelment review
P1	STAR HOSPITAL (2694)	62	Q1+Q8+Q9	32.7%	Suspension review
P1	PRATAP HOSPITAL (3980)	62	Q1	29.9%	Claims review
P1	METRO HOSPITAL NOIDA (1042)	60	Q1+Q2+Q3+Q10	21.8%	Panel + rate audit
P1	PRAYAG HOSPITAL (1065)	—	Q2+Q8+Q9	—	Admission records
P1 NEW	MEDIPLUS HOSP (3613, Nepal)	70	Q10 (foreign)	—	HQ escalation
P1 NEW	YATHARTH SUPER SPEC (3625)	—	Q8	+2,895%	Empanelment review
P1 NEW	MAX SUPER SPEC MOHALI (240)	—	Q8	From zero	Verify empanelment
P2	YASHLOK HOSPITAL (1305)	61	Q6	95.0%	De-panel review
P2	KAMINENI HOSPITALS (177)	60	Q6	55.5%	Claims sampling
P2	MANSAROVAR HOSPITAL (4628)	—	Q3	—	Spot audit
P2	SINGHANIA UNIV HOSP (2844)	62	Q1	28.8%	Claims review
P2	KRUSH DIVINE HOSPITAL (3544)	—	Q9	—	OPD verification
P2	KRISHNA SUPER SPECIALITY (3441)	—	Q8	From zero	Claims verification

Prepared by IIT Kanpur — Data Analytics & Fraud Intelligence Division | 14 May 2026. All findings are based on structured database analysis and must be corroborated with physical audit records before enforcement action. This document supplements Module 1 and should be read in conjunction with it.